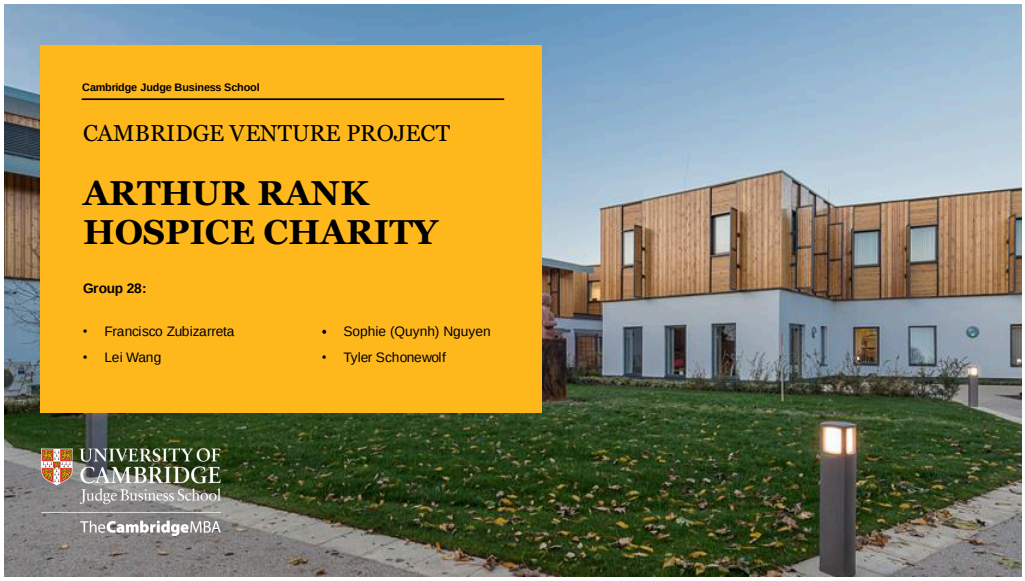


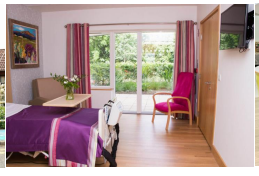
Agenda

1. Introduction to Arthur Rank Hospice Charity
2. Problem statement
3. Methodology
4. Research results and insights
5. Conclusion
6. Recommendations
7. Risk and mitigation plan
8. Action plan

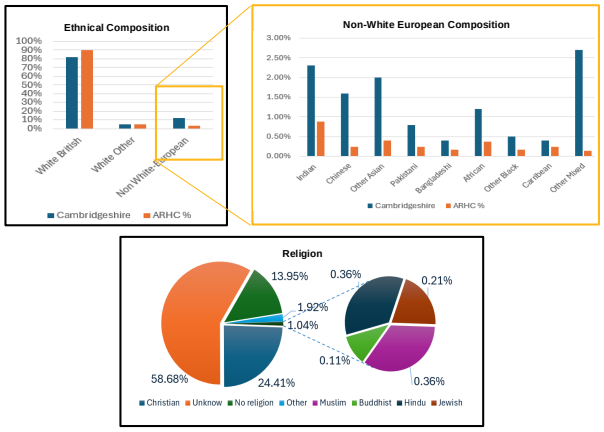
1. Introduction to Arthur Rank Hospice Charity

- **Arthur Rank Hospice Charity (ARHC)** is the only adult hospice that supports people in Cambridgeshire living with an advanced serious illness or other life-limiting condition and those who need end of life care
- **History**: Founded in 1981
- **Size**: 22 beds for in-patient services, serving 3600 patients annually
- **Services:** Apart from the in-patient units, the hospice offers a number of services including but not restricted to: Therapy teams, Lymphoedema Services Specialist care teams, Hospice at Home, Inpatient units
- **Funding:** 60% from the NHS and 40% from retail activities and self-fund raising from donors




2. Problem statement



Source: *ARHC – Patients Report April 2023 to September 2024. Cambridge, U.K.
Office for Health Improvement & Disparities -Palliative and end of life care factsheet: Patterns of care, England 2022

Problem: Gap in Patients Served

- There is a gap to serve Non-White European minorities by ARCH.
- Religion and family values from these groups are considered a major factor on considering palliative care.
- These groups have also can have little visibility on hospice services.

Objective of the project

Conduct research to understand the main blockers for ARHC for engaging with a wider demographic in the Cambridgeshire and provide recommendations on how to overcome these constraints.

3. Methodology and Limitations

Objective

Enable the hospice to better serve Cambridgeshire's diverse population, while carefully considering potential risks associated with implementing the recommended changes

Methodology	**Secondary researches:** Data on demography and population, historical data from hospice, reports and best practices, etc.	**Primary researches:** Interviews with Arthur Ranks' acting CEO, GPs, matrons, staffs and donors, etc.	**Actionable plan** to enhance engagement and strengthen community engagement
Limitations	Most of our primary interviews conducted with white people, without diversity of ethic	More data and secondary researches describe general UK hospice situation, not specifically Cambridgeshire's situation	

Secondary research: Constraints towards Hospice Care

Religion and family values	• Mistrust from patients– Influence of religion (God is the one to decide) • Hospice care conflicting with religion (giving up faith on God's power) • Avoidance of open disclosure due to religious/family values
Demographic and epidemiological issues	• Previous negative care experiences: racism, insensitivity, lack of cultural awareness • Geographical location of hospices; services not available in rural areas • Hospice care conflicting with religion (giving up faith on God's power)
Communication	• Problems with using family/friends as translators • Low number of minority doctors and low availability of training to help professionals
Outcomes / Pain Control	• Pain control– Worse pain outcomes and insufficient availability of opioid medication • Pain severity underestimated (e.g. due to language)
Decision Making	• Less likely to complete advance care planning documents • More likely to mistrust caregivers

4. Results and Insights - Secondary research: Outreach best practices

National Hospice and Palliative Care Organization: Black and African American Outreach Guide

Build partnerships	• Learn about Black and Minority Ethnic Organizations • Connect and co-sponsor events/activities with other community causes
Embrace culture of faith	• The role of the pastor is crucial • Alter hospice perception by celebrating life and living
Host and participate in wider community events	• Engage with and embrace other, presently accepted/integrated community groups
Connect with black and minority ethnic media	• Be aware not to rely too heavily on media portrayals; better to engage with community directly when possible

Secondary research: Community engagement

Barriers to Care	• Limited awareness and knowledge about end-of-life care services. • Language barriers, which impede communication and decision-making. • Cultural and religious insensitivity, resulting in unmet spiritual needs. • Mistrust or misunderstanding of health and care services.
Access and Experiences	• People from BME communities are more likely to die in hospitals rather than in hospices or care homes. • Perceptions that hospices may not accommodate cultural or religious practices deter some individuals.
Engagement Best Practices	• Ensure early identification of individuals approaching the end of life to enable personalized care. • Invest in staff training on cultural, religious, and linguistic competencies. • Collaborate with local communities and advocacy groups to bridge gaps in care delivery.

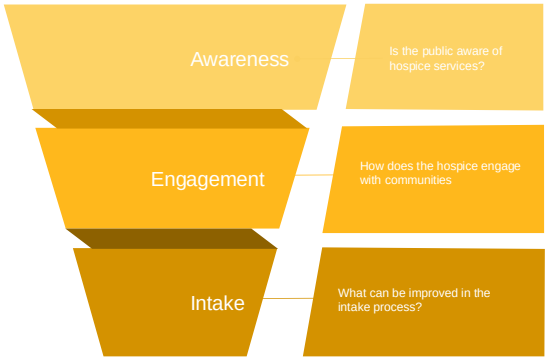
4. Insights - Primary Research

Referral Process	• No specific process optimization has been implemented for referred patients • Rural patients face significant access challenges • Bureaucratic hurdles, especially for Hospice-at-Home services.
Stakeholder Challenges	• Misconceptions about hospice care, particularly in minority communities • Lack of trust in healthcare services among some groups.
Rejection Reasons	• Cultural expectations of caregiving at home • Fear of hospice as solely end-of-life care
Access Gaps	• Geographic disparities in service delivery

5. Conclusion

Key problems		Factors influencing patient's decision	
Perception about hospice	Awareness gap	Cultural and religious beliefs	Some cultures or religions may have specific practices, rituals, or views regarding death and end-of-life care that could impact the choice to pursue hospice care
	Misconception	Family supports and involvement	Family considers their availability of family members, or whether the patient's needs would be better met by a specialized hospice team
	Cultural and religious beliefs	Language issues	If patients face difficulty communicating with hospice staffs, they may be reluctant to choose hospice care
Barriers to access	Location	Location and services	The availability of hospice services around patient area and in a preferred location (at home, in a hospice facility, or a hospital) can influence the decision
	Language	Previous experiences with health system	Past experiences with healthcare providers, particularly those involving end-of-life care, can shape the patient's decision

6. Recommendations



Improve Outreach and Education
- Awareness Campaigns: Early stage/Pathway program
- Empathetic Communication: Leverage personal stories to connect with patients

Foster Community Relationships
- Collaborative relationships with local community, religious leaders and early-stage treatment centers

Enhance Workforce Diversity
- Inclusive Hiring Practices
- Cultural Competency Training

Language Support
- Hire translators or staff members who speak multiple languages
- Install translating function in call or website registration

Streamline Access
- Work with NHS to Simplify Referral Processes
- Expand Services to Rural Areas

7. Risks and Mitigation Plan

Key risks	Description	Impact	Mitigation plan
Community Engagement- Cultural misconceptions about hospice	Fear and reluctance to engage with service	High	- Create culturally sensitive campaigns. - Share testimonials from diverse patients
Community Engagement- Lack of trust in healthcare	Communities avoid hospice services	Medium	- Collaborate with trusted local leaders. - Host community events to build relationships.
Difficulty recruiting diverse staff	Workforce lacks cultural representation	Low	- Launch targeted recruitment initiatives. - Hiring part-time staff or volunteers.
Referral Process - Complex paperwork delays	Delayed access due to change in process, especially urgent cases	Low	- Digitize forms with user-friendly platforms. - Train staff in efficient form handling.

8. Action plan

Short-Term

- Launch outreach campaigns targeting minority groups
- Train staff on cultural competency
- Build relationships with local community leaders

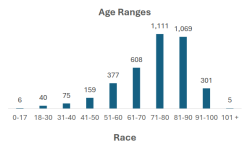
Long-Term

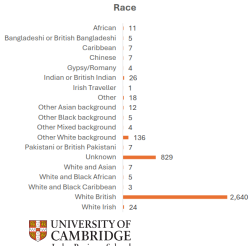
- Work with the NHS to craft a simplified referral system
- Develop satellite services to underserved areas
- Continuously evaluate and improve patient engagement

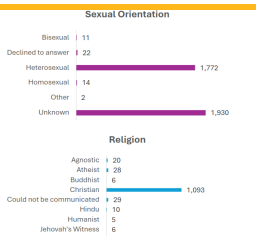
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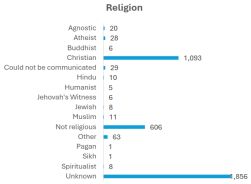
Appendix | ARHC – Patient demographics

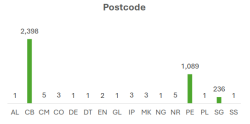
3,751 people known to us between April 2023 and September 2024

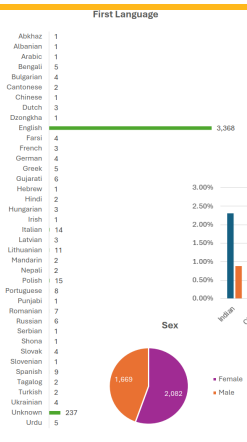


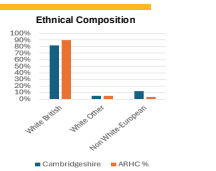


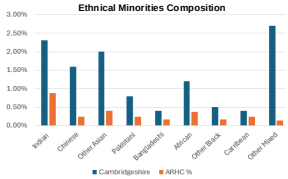




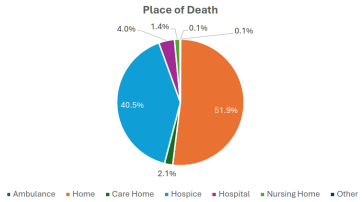


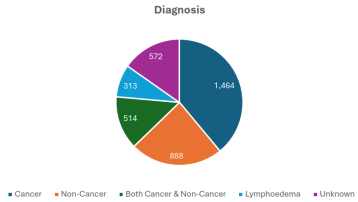


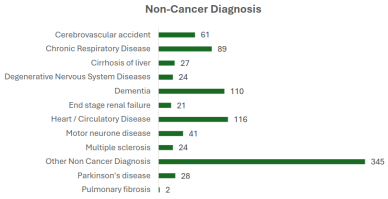


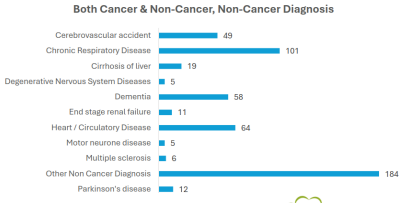


Appendix | ARHC – Main illnesses









Appendix | Causes of death in hospice and palliative care

Number of people who died of or with 4 major conditions in UK 2022

Deaths with the condition	Cancer	Cardiovascular	Dementia	Respiratory	Any of the 4 conditions
Underlying cause of death	139,622	131,278	63,883	59,475	394,258
Only a contributory cause of death	19,473	123,951	29,483	112,810	82,138
Total	**159,095**	**255,229**	**93,366**	**172,285**	**476,396**

Appendix | Comparison of hospices

Country	Model Type	Accessibility	Integration	Key Features	Challenges
United States	Medicare-Driven	85%	90%	• Medicare funding • End-of-life care focus	Excludes early-stage palliative care
United Kingdom	NHS-Integrated	80%	85%	• Integrated with NHS • Community-based	Funding variability, reliance on donations
Singapore	Hybrid (Government/ Charity)	75%	80%	• Emphasis on home care • Government subsidies	Rising demand, limited public awareness
Australia	Community and Inpatient	78%	82%	• Community-focused • Rural outreach	Regional disparities in access
India	NGO-Driven	40%	45%	• NGO-led • Limited resources	Inadequate infrastructure, cultural barriers
South Africa	NGO-Driven	35%	50%	• NGO-led • Focus on HIV/AIDS and cancer care	Severe resource constraints, medication access

- United States: Medicare-driven hospice system; high accessibility.
- United Kingdom: NHS-supported hospice care; early adopters of hospice models.
- Low and Middle-Income Countries (LMICs): Limited services; focus on capacity building
- Africa: 98% of children needing palliative care reside here; significant unmet needs
- Asia: Growing awareness; examples include Singapore's holistic and integrated model